A patient's inpatient journey

From admission to discharge in our current six-dashboard MVP.

Arrival and care

- **1. Find or register the patient.** Admission searches for an existing patient or creates a UHID (permanent patient ID). Each hospital stay gets its own IP admission number.
- **2. Reserve and activate.** Admission selects the bed, treating Doctor and self-pay/insured route. Staff record the required deposit and actual arrival; activation starts room charges. Admin assigns a Nurse; the Nurse starts a care shift.
- **3. Record services and money.** Configured rates calculate room, medicine and performed-service charges. Staff record receipts. Pharmacy records accepted medicine returns; a bill credit and an actual cash refund are separate.
- **4. Verify insurance, if used.** Insurance staff record externally verified policy details, evidence and accepted payer responsibility. Patient share, insurer credit and money received stay separate.

Discharge preparation

- **5. Record the Doctor decision.** The Doctor decides outside the app. The assigned Nurse records the named Doctor, source time, evidence and instructions. Clinical papers can be generated at this stage.
- **6. Complete applicable checks in parallel.** Nurse publishes the checklist. Nursing, Pharmacy and Insurance complete their own checks; Nurse records external diagnostic confirmation when needed. Admin confirms proposed optional N/A only when no relevant work remains.
- **7. Record physical bed vacancy.** After Nursing readiness, Nurse records vacancy, waiting location and continuing-care handover. Room use ends, but the bed remains held.
- **8. Authorize final finance.** Admin/Billing reconciles charges, allocates deposits, records required payments/refunds and settles the patient share. Accepted insurer credit may remain a receivable; unresolved patient dues block ordinary exit.

Papers, exit and bed reuse

- **9. Deliver papers, then confirm exit.** Nurse/Admin records delivery of the exact authorized final papers. The assigned Nurse then confirms actual hospital exit; the admission closes.
- **10. Clean and release the bed.** Admin records authorized cleaning completion before reuse. Later insurer receipts settle the receivable without changing the original exit or saved final papers.

What each dashboard does

Six staff workspaces use the same patient admission and its recorded history.

- **Admin - hospital coordination.** Manages beds, prices, staff, Nurse assignments and availability responsibility; tracks blockers; confirms optional N/A; handles exceptions, final finance and bed turnover.
- **Admission - identity and arrival.** Finds/registers patients, reserves beds, records deposits and actual arrival, and views Doctor availability. Connects insured intake to an insurance case.
- **Nurse - patient readiness and exit.** Works on assigned patients; records external Doctor instructions, checklist, Nursing/diagnostic checks, vacancy and waiting care, paper delivery and actual exit. The designated Nurse updates Doctor availability.
- **Pharmacy - medicine bill reconciliation.** Records patient medicine charges and accepted unused returns, then completes the Pharmacy check. Return credit reduces the bill; Billing records any actual refund.
- **Insurance - verified payer responsibility.** Works on assigned cases; records external verification, decisions, limits, evidence and follow-up, and completes the Insurance check. Accepted cover is distinct from insurer cash received.
- **Billing/Cashier - final money settlement.** Reconciles configured charges, receipts, deposit allocations, refunds and insurer receivables; can authorize final finance, as can Admin, and records later insurer receipts.

How the app keeps the process connected

- **One admission links the records.** Beds/stays, charges, receipts, checks, papers and exit facts are linked in local PostgreSQL. Dashboards show role-appropriate patients, next owners and blockers; the server checks permissions.
- **Changes keep their history.** A bed transfer needs recorded Doctor authority and Nurse handover. Corrections retain addenda/linked adjustments; changes reopen affected approvals. Admin exceptions retain reasons, authority, follow-up and unpaid obligations.
- **External decisions stay with people.** There is no routine Doctor/Lab dashboard in this MVP. Doctors and diagnostic departments provide outside instructions/evidence. Insurance paperwork currently uses manual staff review; no live payer or payment connection is enabled.

A simple fictional example: Isha

- **Bill: ₹3,800.** Patient share ₹800 + accepted insurer credit ₹3,000. From the ₹2,000 patient deposit, ₹800 is allocated and ₹1,200 refunded.
- **Patient settled; insurer cash still ₹0.** After required checks, final finance and paper delivery, Nurse can record exit with ₹3,000 retained as an insurer receivable. A later receipt clears it.

Basis: current app screens, server workflow and local fictional database checked on 18 October 2026. Covers new MVP admissions; older workflow records retain their original history. Example amounts are fictional.